

Human Immunodeficiency Virus Screening and Preexposure Prophylaxis

This Committee Statement was developed by the American College of Obstetricians & Gynecologists' Committee on Clinical Consensus—Gynecology in collaboration with Jessika A. Ralph, MD, MS, Jenell S. Coleman, MD, MPH, and Catherine Cansino, MD, MPH.

Obstetrician–gynecologists (ob-gyns) and other reproductive care professionals are uniquely suited to counsel their patients on human immunodeficiency virus (HIV) screening and HIV preexposure prophylaxis (PrEP) due to their expertise in reproductive health counseling. Ob-gyns and other health care professionals should be obtaining comprehensive sexual health histories, assessing risk behavior, and providing their patients information regarding prevention of sexually transmitted infections such as HIV. When framing their counseling, health care professionals should consider epidemiologic data and HIV prevalence rates for their practice area, and counseling should include discussion of prevention methods. Deciding route of administration and form of PrEP medication should be part of a shared decision-making model between the patient and clinician. By normalizing conversations about HIV prevention, encouraging HIV screening, and ensuring awareness of and access to PrEP, ob-gyns can help reduce disparities in individuals with HIV and those at risk of acquisition.

SUMMARY OF RECOMMENDATIONS AND CONCLUSIONS

On the basis of the principles outlined in this Committee Statement, the American College of Obstetricians & Gynecologists (ACOG) offers the following recommendations and conclusions:

- **Health care professionals should conduct comprehensive, inclusive, and nonjudgmental**

sexual histories during routine wellness visits for all individuals, regardless of age, marital status, or perceived risk.

- **At least one lifetime human immunodeficiency virus (HIV) test should be recommended for all people using an opt-out approach. More frequent testing should be offered to those with ongoing risk factors.**

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- **Health care professionals should be familiar with the local epidemiology of HIV to guide clinical decision making and HIV-prevention counseling.**
- **Health care professionals should proactively initiate discussions about HIV prevention, including preexposure prophylaxis (PrEP), with all sexually active adolescents and adults at increased risk of acquisition.**
- **Health care professionals should prescribe PrEP to patients with increased risk of HIV acquisition and should prescribe PrEP to any individual who requests it.**

BACKGROUND

The human immunodeficiency virus (HIV) epidemic continues to represent a significant public health challenge in the United States. In 2024, there were more than 38,000 new diagnoses reported, with 7,752 (20%) of those among females and with heterosexual contact identified as the predominant mode of transmission within that group (1). The Ending the HIV Epidemic initiative, established in 2019 by the U.S. Department of Health and Human Services, set the goal of 90% reduction in new HIV diagnoses by 2030 using four pillars: 1) early diagnosis, 2) rapid treatment, 3) infection prevention including the use of preexposure prophylaxis (PrEP), and 4) rapid detection of and response to outbreaks (2).

The purpose of this Committee Statement is to update Committee Opinion No. 595, *Preexposure Prophylaxis for the Prevention of Human Immunodeficiency Virus*, and Committee Opinion No. 596, *Routine Human Immunodeficiency Virus Screening*. It is intended to guide and empower obstetrician–gynecologists (ob-gyns) and other health care professionals to counsel their patients on HIV screening and prevention. Although several studies mentioned in this document pertain to ob-gyns, the recommendations on the counseling of HIV prevention and provision of PrEP apply to other reproductive health care professionals. This distinction is acknowledged by the use of “health care professionals” throughout the document. Clinical data on the efficacy of HIV screening tests, treatments, and provision of PrEP are deemed out of scope for this document. Clinical recommendations and clinical best practices regarding HIV screening and provision of PrEP are available in guidance provided by the Centers for Disease Control and Prevention (CDC) and the U.S. Preventative Services Task Force, among other leading medical organizations (see Resources section).

Ob-gyns and other reproductive care professionals are uniquely suited to play important roles in the Ending the HIV Epidemic initiative and are well-positioned to advance the aforementioned pillars. Ob-gyns can increase the awareness of HIV PrEP, which is the use of antiretroviral

medications in people without HIV infection to prevent HIV acquisition. Moreover, ob-gyns provide almost half of all women’s preventative health visits annually, which are visits that often focus on reproductive health–related services, including sexually transmitted infection (STI) screening and prevention (3, 4). Expertise in reproductive health counseling and shared decision making can easily be adapted to include information about HIV screening, HIV prevention, and PrEP (5, 6). Furthermore, individuals want to learn about this information from their ob-gyns and believe them to be trusted sources of information about HIV screening and PrEP (7–9).

Disparities exist with HIV infection among U.S. women and individuals seeking obstetric and gynecologic care. As of 2022, Black women make up 13% of the U.S. population but account for 50% of new HIV cases among women in the United States (10). In 2024, the U.S. HIV diagnosis rate was 21.6 per 100,000 among Black women, 6.4 per 100,000 among Hispanic/Latina women, and 1.8 per 100,000 among White women (1). Young people aged 13–24 years also made up 18% of new HIV diagnoses in 2024 and are the most likely group to be living with undiagnosed HIV (1). History of intimate partner violence is significantly associated with HIV infection, and transgender individuals are at high risk of HIV acquisition (11–13). Furthermore, all of these groups experience barriers to PrEP uptake, including patient and health care professional knowledge, stigma, cost, racism, poverty, access, and medical distrust (14–16). A 2015 estimate by Huang et al showed that fewer than 5% of PrEP users were women, accounting for only 2.1% of heterosexual women with indications for PrEP, and that White men and women were nearly six times more likely to be prescribed PrEP than Black men and women (17). Smith et al have similarly demonstrated that, although Black individuals account for 40% of people with indications for PrEP, utilization among this group is low (9.1% of males and 17.1% of females) (18). Additional guidance for ob-gyns on ways to provide care with cultural humility to better address health disparities can be found in ACOG Committee Statement No. 10, *Racial and Ethnic Inequities in Obstetrics and Gynecology* (19).

COUNSELING CONSIDERATIONS

Health care professionals should conduct comprehensive, inclusive, and nonjudgmental sexual histories during routine wellness visits for all individuals, regardless of age, marital status, or perceived risk.

Ob-gyns provide a large share of women’s preventative care visits (3), and an annual wellness visit with an ob-gyn or other gynecologic care professional may be the only interaction a patient may have with the health care system during a given year. It is therefore imperative that ob-gyns

Box 1. The Five P's Approach for Health Care Professionals Obtaining Sexual Histories

1. Partners

- Are you currently having sex of any kind?
- What is the gender(s) of your partner(s)?

2. Practices

- To understand any risks for sexually transmitted infections (STIs), I need to ask more specific questions about the kind of sex you have had recently.
- What kind of sexual contact do you have or have you had?
- Do you have vaginal sex, meaning "penis in vagina" sex?
- Do you have anal sex, meaning "penis in rectum/anus" sex?
- Do you have oral sex, meaning "mouth on penis/vagina"?

3. Protection from STIs

- Do you and your partner(s) discuss prevention of STIs and human immunodeficiency virus (HIV)?
- Do you and your partner(s) discuss getting tested?
- For condoms:
 - What protection methods do you use? In what situations do you use condoms?

4. Past history of STIs

- Have you ever been tested for STIs and HIV?
- Have you ever been diagnosed with an STI in the past?
- Have any of your partners had an STI?

Additional questions for identifying HIV and viral hepatitis risk:

- Have you or any of your partner(s) ever injected drugs?
- Is there anything about your sexual health that you have questions about?

5. Pregnancy intention

- Do you think you would like to have (more) children in the future?
- How important is it to you to prevent pregnancy (until then)?
- Are you or your partner using contraception or practicing any form of birth control?
- Would you like to talk about ways to prevent pregnancy?

Modified from Centers for Disease Control and Prevention. STI and HIV infection risk assessment. In: Sexually transmitted infections treatment guidelines, 2021. Accessed April 24, 2026. <https://www.cdc.gov/std/treatment-guidelines/clinical-risk.htm>

and other health care professionals obtain a comprehensive sexual health history, assess risk behavior, and provide information regarding prevention of STIs, including HIV, to all their patients. Obtaining a sexual history should be gender-inclusive and done with respect and compassion. The CDC's Five P's approach (detailed in Box 1) provides a framework for this discussion and is intended to gather information about 1) Partners, 2) Practices, 3) Protection from STIs, 4) Past history of STIs, and 5) Pregnancy intention (20). Additional guidance about obtaining a sexual history in adolescent patients can be found in ACOG Committee Opinion No. 811, *The Initial Reproductive Health Visit*.

Health care professionals can refer to Box 2 for a table of behaviors associated with increased risk of STI acquisition, including acquisition of HIV.

At least one lifetime HIV screening test should be recommended for all people using an opt-out approach. More frequent testing should be offered to those with ongoing risk factors.

All individuals aged 15–65 years should be screened at least once for HIV infection, and all pregnant patients should be screened at least once every pregnancy (21, 22); however, as of 2019, fewer than 40% of adults have ever been tested for HIV infection (23). Among individuals

Box 2. Risk Factors for Sexually Transmitted Infection Acquisition (Including Human Immunodeficiency Virus [HIV] Infection)*

History of an STI in the past year or history of gonorrhea or syphilis within the past 6 months

Inconsistent use of condoms during vaginal or anal intercourse

Known HIV-positive partner

Multiple sexual partners of unknown HIV status

Use of injection drugs

Exchange of sex for money or housing (transactional sex)

Living in a correctional facility

STI, sexually transmitted infection; HIV, human immunodeficiency virus.

*Adapted from Centers for Disease Control and Prevention, U.S. Public Health Service. Preexposure prophylaxis for the prevention of HIV infection in the United States—2021 update: a clinical practice guideline. Accessed April 24, 2026. <https://stacks.cdc.gov/view/cdc/112360>

with risk factors for HIV infection, only a quarter reported being offered testing in the previous year if they had a visit with a health care professional (24). Although physicians report certain barriers to offering testing, such as time pressure during the health care appointment, cost concerns, and perceived patient discomfort with discussing HIV (25–27), patients have reported that the most important reason they underwent HIV screening was because their health care professional recommended it (7–9, 28, 29).

Health care professionals should be familiar with the local epidemiology of HIV to guide clinical decision making and HIV-prevention counseling.

Epidemiologic awareness should frame how ob-gyns and other health care professionals discuss HIV prevention with their patients. In high-prevalence settings, health care professionals can justifiably emphasize that many people in the community are living with HIV, that there is ongoing transmission, and that the risk is real even for patients who do not identify with the traditional risk categories. In lower-incidence settings, the conversation still may need to focus on individual behaviors and exposures, but epidemiologic context reminds both health care professionals and patients that low risk is a continuum and does not equate to absence of risk. For example, in 2024, the U.S. South accounted for 51% of HIV diagnoses. In contrast, the Midwest reported the lowest rates in the nation (14%), which can help to calibrate discussions around risk (1). Health care professionals are encouraged to consult free online data tools and to check with their local health departments to learn more about the incidence rates of HIV specific to the communities and areas they serve (see Resources).

Health care professionals should proactively initiate discussions about HIV prevention, including preexposure prophylaxis (PrEP), with all sexually active adolescents and adults at increased risk of acquisition.

All sexually active adolescents and adults who are at increased risk for STIs should receive behavioral counseling for the prevention of STIs (30). Counseling should include discussion of prevention methods such as reducing the number of sexual partners, appropriate barrier method usage, STI screening, and information about PrEP (21, 30). Behavioral counseling may be brief and completed by the health care professional during the visit, and such counseling can also be conducted effectively through asynchronous means such as video or group presentation (21, 31). The CDC further recommends informing all sexually active adolescents and adults about PrEP for HIV prevention, both in the context of risk assessment and to encourage them to discuss it within their social networks, particularly among those who may benefit from PrEP use (32).

Health care professionals should prescribe PrEP to patients with increased risk of HIV acquisition and should prescribe PrEP to any individual who requests it.

There are multiple methods for PrEP for cisgender women, detailed in Table 1 (32). Currently, injectable PrEP is available for administration only by a health care professional; ob-gyns and other reproductive health care professionals should counsel their patients about injectable PrEP availability and refer them accordingly. Deciding on route of administration and form of PrEP medication should be part of a shared decision-making model between the patient and their health care professional. As with other medications, health care

Table 1. Preexposure Prophylaxis Medications for Cisgender Women, With Suggested Laboratory Monitoring Schedules

Formulation	Generic Name	Brand Name	Dosage	Frequency	Laboratory Monitoring Test	Suggested Testing Schedule										
						At Initiation	At 1 mo	At 2 mo	At 3 mo	At 4 mo	At 6 mo	At 8 mo	At 9 mo	At 10 mo	At 12 mo	On Cessation of Therapy
Oral PrEP	Emtricitabine and tenofovir disoproxil fumarate	Truvada	200 mg emtricitabine/300 mg tenofovir disoproxil fumarate	Once daily	HIV	x	—	—	x	—	—	—	x	—	x	x
					Creatinine	x	—	—	—	—	x	—	—	—	—	x
					Syphilis	x	—	—	—	—	x	—	—	—	x	—
					Gonorrhea	x	—	—	—	—	x	—	—	—	x	—
					Chlamydia	x	—	—	—	—	x	—	—	—	x	—
					Hepatitis B	x	—	—	—	—	—	—	—	—	—	—
					Hepatitis C	x	—	—	—	—	—	—	—	—	—	—
					Pregnancy test	x	—	—	x	—	—	—	x	—	x	—
Injectable PrEP	Cabotegravir*	Apretude	600-mg intramuscular injection	2 times monthly at initiation, then once every 2 mo	HIV	x	x	x	—	x	x	x	—	x	x	x
					Creatinine	x	—	—	—	—	—	—	—	—	—	—
					Syphilis	x	—	—	—	—	x	—	—	—	x	—
					Gonorrhea	x	—	—	—	—	x	—	—	—	x	—
					Chlamydia	x	—	—	—	—	x	—	—	—	x	—
					Hepatitis B	x	—	—	—	—	—	—	—	—	—	—
					Hepatitis C	x	—	—	—	—	—	—	—	—	—	—
					Pregnancy test	x	—	—	—	x	—	x	—	—	x	—
	Lenacapavir	Yeztugo	927-mg intramuscular injection	Every 6 mo	HIV	x	—	—	—	—	x	—	—	—	x	x
					Creatinine	x	—	—	—	—	—	—	—	—	—	—
					Syphilis	x	—	—	—	—	—	—	—	—	x	x
					Gonorrhea	x	—	—	—	—	—	—	—	—	x	x
					Chlamydia	x	—	—	—	—	—	—	—	—	x	x
					Hepatitis B	x	—	—	—	—	—	—	—	—	—	—
					Hepatitis C	x	—	—	—	—	—	—	—	—	—	—
					Pregnancy test	x	—	—	—	—	x	—	—	—	x	—

PrEP, preexposure prophylaxis; HIV, human immunodeficiency virus.

Data adapted from Centers for Disease Control and Prevention, U.S. Public Health Service. Preexposure prophylaxis for the prevention of HIV infection in the United States—2021 update: a clinical practice guideline. Accessed April 24, 2026. <https://stacks.cdc.gov/view/cdc/112360>; and Wood BR, Budak JZ, Cannon CA, Spach DH. Clinician's information guide: lenacapavir for HIV PrEP. In: Spach DH, editor. National HIV PrEP curriculum. University of Washington Infectious Diseases Education & Assessment Program. October 28, 2025. Accessed November 10, 2025. <https://www.hivprep.uw.edu/page/clinical/guides#lenacapavir>.

*Optional: 30 mg oral cabotegravir daily for first 28 days of therapy.

professionals should be aware of contraindications and adverse drug interactions associated with specific methods of PrEP. Information on identifying where injectable PrEP is available is included in the Resources section.

PrEP should be prescribed to adults and adolescents who weigh at least 35 kg or 77 lbs with the high-risk behaviors listed in Box 2 (33). The CDC further recommends prescribing PrEP to anyone who requests it, acknowledging that people may request it because of concern over HIV acquisition yet hesitate to disclose sexual or injection behaviors (32).

A negative HIV test result should be documented within the week before initiating PrEP, and patients should also undergo testing for gonorrhea, chlamydia, and trichomoniasis. HIV screening should continue at regular intervals

throughout the duration of PrEP use, regardless of route, and patients should be counseled to continue to use barrier methods to prevent other STIs because PrEP does not protect against those infections. For patients who do not desire pregnancy, any contraceptive method can be used with either route of PrEP; contraceptive efficacy is not affected by PrEP use. For patients desiring pregnancy and who are at high risk of HIV acquisition, oral PrEP has good safety data in pregnant and lactating individuals (32, 34).

Reviewing how to manage side effects and how to handle missed doses and helping patients identify possible reasons that they may struggle to adhere to their regimen and how to address those reasons all help improve adherence. Fewer than 10% of people who take oral PrEP report headache, nausea, or abdominal

discomfort within the first month of use (35, 36). Over-the-counter medications should alleviate these temporary symptoms. With regards to missed dosing, the CDC stipulates that people should take the missed dose as soon as they remember it, unless it is almost time for the next dose (32). If it is almost time for the next dose, patients should skip the missed dose and continue with the regular dosing schedule (37).

Medication adherence for oral PrEP users is key to achieving maximum protection against HIV. An adherence rate of 85% or greater is needed to maintain protective drug levels in vaginal tissue (38). Injectable PrEP has demonstrated improved adherence and therefore improved reduction in HIV incidence.

For injectable PrEP, efficacy is similarly dependent on consistent dosing, with long-acting cabotegravir demonstrating 93% on-time injection coverage and an 88% reduction in HIV incidence among cisgender women. Both adherence and reduction in HIV were notably lower in the oral tenofovir disoproxil fumarate-emtricitabine group (39). Additionally, twice-yearly lenacapavir showed 100% efficacy with zero incident infections when administered as scheduled (40).

Available evidence on options for PrEP continue to evolve, and both new drug-delivery systems and drug combinations are being evaluated. Studies are ongoing for a vaginal ring containing dapirivine (41), a PrEP implant (42), and an oral pill (43) containing both PrEP and hormonal contraception. Given the skills of ob-gyns to counsel regarding hormonal contraception in all of these routes, the specialty is well-suited to continue to prescribe these new methods of PrEP as they become available.

CONCLUSION

The continued transmission of HIV is preventable, and PrEP is an underutilized prevention tool. Ob-gyns and other reproductive health care professionals are uniquely suited to lead the integration of HIV-prevention methods, inclusive of PrEP, into comprehensive sexual and reproductive health care. By normalizing conversations about HIV prevention, encouraging HIV screening, and ensuring awareness of and access to PrEP, ob-gyns can help reduce disparities among individuals with HIV and those at risk of acquisition and fulfill the promise of ending the HIV epidemic in the United States.

RESOURCES

These materials are for information purposes only and are not meant to be comprehensive. Referral to these resources does not imply ACOG's endorsement of the organization, the organization's website, or the content of the resource. The resources may change without notice.

HIV Epidemic Data

- Ending the HIV Epidemic in the U.S.: AHEAD | America's HIV Epidemic Analysis Dashboard
- Center for AIDS Research at Emory University (CFAR): AIDSVu

General PrEP Information

- Centers for Disease Control and Prevention: Preexposure Prophylaxis for the Prevention of HIV Infection in the United States—2021 Update: A Clinical Practice Guideline (View PDF)
 - The updated guideline and supplement reflect the latest science and are intended to help clinicians effectively prescribe all U.S. Food and Drug Administration–approved PrEP medications to patients and increase PrEP use among all people who could benefit.
- U.S. Preventative Services Task Force: Recommendation: Prevention of Acquisition of HIV: Preexposure Prophylaxis (2023) (View PDF)
- National Clinician Consultation Center:
 - PrEP Guidelines & Resources
 - HIV/AIDS Management
 - The National Clinician Consultation Center provides clinicians of all experience levels prompt, expert responses to questions about managing HIV and acquired immunodeficiency syndrome (AIDS), perinatal HIV, preexposure prophylaxis, and bloodborne pathogen exposures. They provide online and phone-based consultation in service areas, including testing and prevention, treatment, and postexposure prophylaxis. All of the services are cost-free and confidential.
- National HIV PrEP Curriculum: <https://www.hivprep.uw.edu/>
 - The National HIV PrEP Curriculum is a repository of educational resources for health care professionals looking to learn more about managing and initiating PrEP for their patients.

Locating PrEP Prescribers

- Centers for Disease Control and Prevention: National Prevention Information Network
 - The National Prevention Information Network provides a comprehensive directory of organizations in the United States that offer PrEP to prevent HIV infection.

Information for Patients

- Centers for Disease Control and Prevention: #ShesWell: PrEP for Women | HIV Prevention | Let's Stop HIV Together | CDC
 - This patient site answers common questions from patients regarding PrEP, such as its efficacy, how to

start the conversation about PrEP with your health care clinician, and use of PrEP while using birth control. It offers tips on what questions women and other people who have receptive vaginal sex can ask their health care clinicians about PrEP.

- U.S. Department of Health and Human Services. HIV.gov, Pre-Exposure Prophylaxis
 - HIV.gov provides patient-focused information on PrEP, including its safety, what drugs are approved for PrEP, and how to obtain PrEP, as well as insurance coverage and patient-assistance programs.

Coverage

- National Alliance of State and Territorial AIDS Directors: State PrEP Assistance Programs
 - The site provides a list of state programs that provide medication assistance to PrEP candidates.

USE OF LANGUAGE

The American College of Obstetricians & Gynecologists (ACOG) recognizes and supports the gender diversity of all patients who seek obstetric and gynecologic care. In this document, authors seek to use gender-inclusive language or gender-neutral language. When describing research findings, this document uses gender terminology reported by investigators. To review ACOG's policy on inclusive language, see <https://www.acog.org/clinical-information/policy-and-position-statements/statements-of-policy/2022/inclusive-language>.

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